

DISCHARGE SUMMARY

Patient	Ananya Krishnan	Age/Sex	34 Years / Female
IP No.	FMH-2024-03219	Ward	Pulmonology — Room 208
Admitted	05-Aug-2024	Discharged	08-Aug-2024
Consultant	Dr. Suresh Pillai, MD Pulmonology	Dept.	Pulmonology

DIAGNOSIS

- Acute exacerbation of Bronchial Asthma — moderate severity
- Generalised Anxiety Disorder — known, on treatment
- Allergic Rhinitis — seasonal

HISTORY

34-year-old female software engineer, known asthmatic since age 12. Presented with worsening breathlessness and wheeze for 2 days following exposure to dust during home renovation. Increased use of rescue inhaler (6-8 puffs/day). No fever. Known anxiety disorder — on Escitalopram 10mg OD.

ALLERGY HISTORY

ALLERGY: Aspirin — triggers severe bronchospasm (Samter's triad suspected). AVOID NSAIDs.
ALLERGY: Dust mites, pollens, cat dander — environmental triggers.

INVESTIGATIONS

Test	Result	Reference	Status
Peak Flow Rate (on admission)	210 L/min	> 400 L/min	LOW — 52% predicted
Peak Flow Rate (at discharge)	380 L/min	> 400 L/min	Improved
SpO2 (on admission)	91%	95–100%	LOW
SpO2 (at discharge)	98%	95–100%	NORMAL
Total IgE	420 IU/mL	< 100 IU/mL	HIGH — atopic
Absolute Eosinophil Count	620 cells/ μ L	100–400	HIGH
Chest X-ray	Hyperinflation, no consolidation	—	—

TREATMENT GIVEN

- Nebulisation: Salbutamol + Ipratropium every 4 hours
- IV Hydrocortisone 100mg TDS for 48 hours → oral Prednisolone 40mg taper
- Oxygen therapy: 2L/min via nasal prongs
- Continued Escitalopram 10mg OD

DISCHARGE MEDICATIONS

Medication	Dose	Frequency	Notes
Budesonide/Formoterol inhaler	200/6mcg	2 puffs BD	Controller — long term

Salbutamol inhaler	100mcg	2 puffs SOS	Rescue — use when needed
Prednisolone	20mg → taper	OD for 5 days	Tapering dose
Montelukast	10mg	Once at night	Leukotriene blocker
Escitalopram	10mg	Once daily	Continue for anxiety
Loratadine	10mg	Once daily	Allergic rhinitis

FOLLOW-UP

- Pulmonology OPD in 2 weeks — spirometry reassessment
- Psychiatry OPD in 4 weeks — anxiety review
- Avoid trigger exposure: dust, smoke, NSAIDs, Aspirin
- Asthma action plan given. Peak flow diary to be maintained.